Initial Management of Noncastrate Advanced, Recurrent, or Metastatic Prostate Cancer: ASCO Guideline Update: ASCO Guideline Q and A

Katherine S. Virgo, PhD, MBA[1]; R. Bryan Rumble, MSc[2]; and James A. Talcott, MD, SM[3]

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ASCO recently published an updated guideline on the initial management of noncastrate advanced, recurrent, or metastatic prostate cancer.[1] This companion Q&A article addresses some of the questions that clinicians may have in implementing the guidelines. Figure 1 displays the recommendations in algorithm format. Figure 2 presents the recommendations relevant to the material covered in this Q&A.

QUESTION: WHAT ARE THE PREFERRED TREATMENT OPTIONS FOR PATIENTS WITH DE NOVO METASTATIC NONCASTRATE PROSTATE CANCER WHO ARE CHEMOTHERAPY CANDIDATES?

There are three treatment options. First, among patients with Eastern Cooperative Oncology Group (ECOG) performance status (PS) scores of 0 or 1, darolutamide plus docetaxel plus androgen deprivation therapy (ADT) demonstrated improved overall survival (OS) compared with placebo plus docetaxel plus ADT.[2] Overall mortality was 32% lower in the treatment group than in the control group, despite subsequent systemic therapies, primarily other androgen receptor pathway inhibitors (75.6%). Other benefits included significantly longer time to the development of castration-resistant prostate cancer (hazard ratio [HR], 0.36; 95% CI, 0.30 to 0.42; $P < .001$), pain progression (HR, 0.79; 95% CI, 0.66 to 0.95; $P = .01$), and institution of subsequent systemic antineoplastic therapy, primarily alternative androgen receptor pathway inhibitors (HR, 0.39; 95% CI, 0.33 to 0.46; $P < .001$).

Second, among patients with ECOG PS scores of 0, 1, or 2 if due to bone pain only, abiraterone plus prednisone or prednisolone plus docetaxel plus ADT significantly improved radiographic progression-free survival (rPFS; HR, 0.50; 99.9% CI, 0.34 to 0.71; $P < .0001$), OS (HR, 0.75; 95.1% CI, 0.59 to 0.95; $P < .017$), and castration-resistant prostate cancer (CRPC)–free survival (HR, 0.38; 95% CI, 0.31 to 0.47; $P < .0001$).[3] In addition, among the subgroup of patients with high-volume metastatic burden per CHAARTED[4], OS was significantly improved (HR, 0.72; 95.1% CI, 0.55 to 0.95; $P < .019$) as was rPFS (HR, 0.47; 99.9% CI, 0.30 to 0.72; $P < .0001$).

Third, in resource-constrained circumstances, such as lack of access to androgen receptor–targeted agents (ARTAs) or uninsured status, docetaxel plus ADT is an option. Patients should be made aware that doublet therapy (docetaxel plus ADT) has proven inferior OS compared with triplet therapy, such as abiraterone and prednisone or prednisolone plus docetaxel plus ADT.

QUESTION: WHAT ARE THE PREFERRED TREATMENT OPTIONS FOR PATIENTS WITH DE NOVO METASTATIC NONCASTRATE PROSTATE CANCER WHO ARE NOT CHEMOTHERAPY CANDIDATES BUT ARE CONSIDERED HIGH RISK PER THE LATITUDE TRIAL?

For these patients, ADT plus abiraterone plus prednisone or prednisolone is recommended.

QUESTION: HOW DID THE RESULTS OF THE ENZAMET AND ARCHES TRIALS REGARDING ENZALUTAMIDE AFFECT THE GUIDELINE UPDATE?

The results of the ENZAMET[5] and ARCHES[6] trials did not change the 2021 recommendation for enzalutamide other than reporting long-term results from these trials that were not available in 2021.[7]

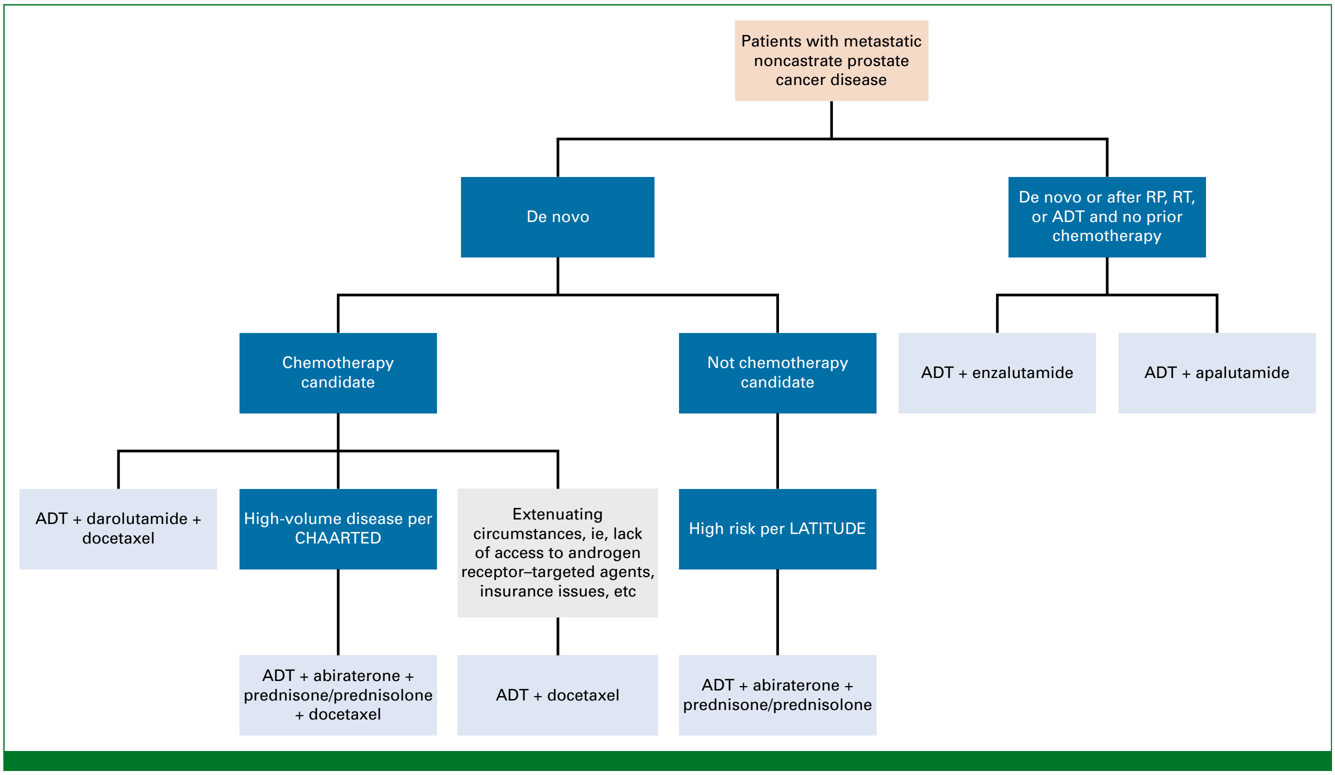

FIG 1. Initial management of noncastrate metastatic prostate cancer algorithm. ADT, androgen deprivation therapy; HVD, high-volume disease; RP, radical prostatectomy; RT, radiotherapy.

QUESTION: WHAT ARE THE PREFERRED TREATMENT OPTIONS FOR PATIENTS WITH LOW-VOLUME DE NOVO METASTATIC NONCASTRATE PROSTATE CANCER?

ADT plus enzalutamide should be offered to patients with metastatic noncastrate prostate cancer including both those with de novo metastatic disease and those who have received prior therapies for localized disease, such as radical prostatectomy or radiotherapy. Enzalutamide has long-term survival benefits for those with both low-volume disease or high-volume disease and no prior docetaxel use.

OS data for patients with low-volume de novo metastatic noncastrate prostate cancer from PEACE-1[3] are still too immature to justify recommending abiraterone-based triplet therapy (abiraterone and prednisone or prednisolone plus docetaxel plus ADT).

For patients with low-volume metastatic disease as defined per CHAARTED[4] who are candidates for chemotherapy, docetaxel plus ADT should not be offered because of lack of sufficient evidence.

QUESTION: IF PATIENTS WITH DE NOVO METASTATIC NONCASTRATE PROSTATE CANCER ARE TO RECEIVE ADT PLUS AN ARTA, DO THEY REALLY STILL NEED DOCETAXEL?

An important question is whether docetaxel is really still necessary in the treatment of patients with metastatic noncastrate prostate cancer. No phase III clinical trials have yet compared, for example, darolutamide plus ADT or abiraterone plus ADT versus docetaxel plus ADT. This as yet unanswered question will likely be the focus of the next guideline update once sufficient randomized clinical trial data become available.

QUESTION: THE GUIDELINE PROVIDES DETAILED INFORMATION ABOUT THE EFFICACY OF THE TREATMENT OPTIONS. GIVEN THE GROWING NUMBER OF PROVEN TREATMENTS, WHAT OTHER ELEMENTS MAY ENTER INTO THE TREATMENT DECISION?

Patient-centered cancer care is widely endorsed but difficult to achieve. Cancer is a frightening diagnosis that forces emotion into rational conversation; both cancer and its treatment are complex and technical, and the information asymmetry between provider and patient is enormous. Simply describing the patient's diagnosis, staging, prognosis, the goal of treatment, and side effects is a huge information transfer. Even if efficacious alternative treatments exist, many patients may be reluctant to weigh in with their preferences: "What would you do, doctor?"

Still, the available treatment options for noncastrate metastatic prostate cancer make shared decision making worthwhile. The available options differ in mechanism (cytotoxic *v* hormonal), duration (18 weeks *v* to progression), efficacy (HR and comparators), prominent side

Recommendation	Type	Evidence Quality	Strength
1.1. For patients with metastatic noncastrate prostate cancer with high-volume disease (HVD) as defined per CHAARTED[4] (four or more bone metastases, one or more of which is outside of the spine or pelvis, and/or the presence of any visceral disease) who are candidates for treatment with chemotherapy but are unwilling or unable to receive triplet therapy (eg, because of insurance constraints), docetaxel plus ADT should be offered.	EB	H	S
Practical Information for Recommendation 1.1: Patients should be made aware that doublet therapy (docetaxel plus ADT) has proven inferior OS compared with triplet therapy, such as abiraterone and prednisone plus docetaxel plus ADT.			
1.11. For patients with de novo metastatic noncastrate prostate cancer with HVD as defined per CHAARTED[4] who are being offered ADT plus docetaxel chemotherapy, triplet therapy (abiraterone and prednisone plus ADT and docetaxel) should be offered per PEACE-1.[3] Abiraterone and prednisone plus ADT and docetaxel demonstrated significant OS and radiographic progression-free survival benefits compared with ADT and docetaxel alone for patients with HVD.	EB	H	S (for patients with HVD as defined per CHAARTED)[2]
Practical Information for Recommendation 1.11: OS data for patients with low-volume de novo metastatic noncastrate prostate cancer from PEACE-1[3] are still too immature to justify recommending abiraterone-based triplet therapy (abiraterone and prednisone plus ADT and docetaxel).			
1.15. For patients with de novo metastatic noncastrate prostate cancer who are being offered ADT plus docetaxel chemotherapy, triplet therapy (darolutamide plus ADT and docetaxel) should be offered per ARASENS.[2] Compared with placebo plus ADT and docetaxel, darolutamide plus ADT and docetaxel demonstrated significant OS benefits, in addition to significantly longer times to castration-resistant prostate cancer, pain progression, first skeletal event, and initiation of subsequent systemic antineoplastic therapy.	EB	H	S
1.16. The recommended regimen for patients with metastatic noncastrate prostate cancer treated with darolutamide, docetaxel and ADT is darolutamide (600 mg as two 300 mg tablets orally with food) twice daily (to a total daily dose of 1200 mg) with ADT. Docetaxel administration (75 mg/m^2) should begin within 6 weeks of the first dose of darolutamide. Docetaxel should be administered intravenously once every 3 weeks for up to six cycles.	EB	H	S
Practical Information for Recommendations 1.15 and 1.16: Discussions with patients should include the cost of darolutamide treatment compared with other options, such as abiraterone.			
1.7. ADT plus enzalutamide should be offered to patients with metastatic noncastrate prostate cancer including both those with de novo metastatic disease and those who have received prior therapies, such as radical prostatectomy or radiotherapy for localized disease. Enzalutamide plus ADT has demonstrated short-term survival benefits (prostate-specific antigen progression-free, clinical progression-free, and overall) when compared with ADT alone for patients with metastatic noncastrate prostate cancer as a group per ENZAMET.[5] Enzalutamide also has long-term survival benefits overall, for those with LVD or HVD, and those with LVD or HVD and no docetaxel use.[8] Per ARCHES,[6] enzalutamide significantly improved time to first subsequent antineoplastic therapy in addition to survival benefits overall and among those with HVD, no prior docetaxel and previous use of ADT or orchiectomy.[6]	EB	H	S
1.8. The recommended regimen for patients with metastatic noncastrate prostate cancer is enzalutamide (160 mg once per day) with ADT.	EB	H	S
Practical Information for Recommendations 1.8: Discussions with patients should include the cost of enzalutamide treatment compared with other options, such as abiraterone.			

FIG 2. Summary of guideline recommendations covered in this Q&A. ADT, androgen deprivation therapy; EB, evidence-based; H, high; HVD, high-volume disease; LVD, low-volume disease; OS, overall survival; S, strong.

effects (alopecia, nausea, fatigue, neutropenia, and peripheral neuropathy), and cost, both drug price and out-of-pocket costs. After discussion, patients may identify a preferred treatment option, better understand the clinical path ahead, and begin team building between clinical staff, patients, and their caregivers. The update mentions discussing costs in the text and includes price information in a table. The trial descriptions include additional information about treatments. Discussing them may bring providers and patients together on a treatment path they jointly selected.

The cytotoxic treatments and newer hormonal agents differ substantially between the drug classes and within-class agents. Docetaxel produces common cytotoxic side effects, such as nausea and cytopenia, and more specific ones, such as alopecia and peripheral neuropathy. However, most of these side effects resolve after the defined treatment course, and docetaxel is inexpensive. The symptoms of hormonal treatments appear modest but continue because treatment continues. Hormonal agents also differ in cost, with generic abiraterone available and modest price differences between the newer beta-blocking agents.

AFFILIATIONS

[1]Rollins School of Public Health, Department of Health Policy and Management, Emory University, Atlanta, GA
[2]American Society of Clinical Oncology, Alexandria, VA
[3]NantHealth, Inc, Culver City, CA

CORRESPONDING AUTHOR

American Society of Clinical Oncology, 2318 Mill Rd, Ste 800, Alexandria, VA 22314; e-mail: guidelines@asco.org.

DISCLAIMER

This Q&A is derived from recommendations in Initial Management of Noncastrate Advanced, Recurrent, or Metastatic Prostate Cancer: ASCO Guideline Update. This document is based on an ASCO Guideline and is not intended to substitute for the independent professional judgment of the treating physician. Practice guidelines do not account for individual variation among patients. This Q&A does not purport to suggest any particular course of medical treatment. Use of the guideline and this Q&A are voluntary. Please refer to the complete guideline to understand the full recommendations.[7]

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AUTHOR CONTRIBUTIONS

Conception and design: All authors
Administrative support: R. Bryan Rumble
Collection and assembly of data: All authors
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James A. Talcott
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